Prodigy Anesthesia Quick Review

Esophagoscopy/Gastroscopy

What are the most common gastrointestinal endoscopic procedures?	Colonoscopy, esophagogastroduodenoscopy (EGD), sigmoidoscopy, and endoscopic retrograde cholangiopancreatography (ERCP). *Longnecker DE, Newman MF, Brown DL, Zapol WM. Anesthesiology. 2nd ed. New York: McGraw-Hill; 2012: 1268.*
What are the anesthetic implications of the patient undergoing colonoscopy?	Patients may present with hemodynamic instability due to hypovolemia from the bowel preparations and prolonged NPO status. Anemia may also be present in patients undergoing evaluation of gastrointestinal bleeding. *Longnecker DE, Newman MF, Brown DL, Zapol WM. Anesthesiology. 2nd ed. New York: McGraw-Hill; 2012: 1269.*
Why is anesthesia frequently involved in the performance of colonoscopies?	Although the procedure has historically been performed under mild sedation administered by the endoscopist, there has been an increase in resistance by patients to undergo the procedure without a deeper level of sedation due to fear of discomfort. *Longnecker DE, Newman MF, Brown DL, Zapol WM. Anesthesiology. 2nd ed. New York: McGraw-Hill; 2012: 1269.*
What are the potential complications of an upper endoscopy that are significant to the anesthetist?	Upper airway obstruction complicated by limited access to the face and head, perforation of the upper GI tract, bleeding, and bradycardia or other arrhythmias arising from distention of the GI tract. *Longnecker DE, Newman MF, Brown DL, Zapol WM. Anesthesiology. 2nd ed. New York: McGraw-Hill; 2012: 1269.*
How is the patient typically positioned for an endoscopic retrograde cholangiopancreatography (ERCP)?	The patient is either in an extreme lateral or prone position *Longnecker DE, Newman MF, Brown DL, Zapol WM. Anesthesiology. 2nd ed. New York: McGraw-Hill; 2012: 1269.*
Is pregnancy a contraindication to esophagogastroduodenoscopy?	No. EGD has been performed successfully in pregnant patients. Consideration should be given to the drugs administered to the parturient as they may pass to the fetus, but propofol and fentanyl have been shown to be safe. Midazolam crosses the placenta and results in fetal depression, so it is not indicated for pregnant patients. *Nagelhout JJ, Plaus KL. Nurse Anesthesia. 5th ed. St. Louis, MO: Elsevier Saunders Company; 2014: 1271-1272.*
Does endoscopy require antibiotic prophylaxis for patients at risk for endocarditis?	Because EGD involves the instrumentation of the oropharynx and trauma to the mucosa is a possibility (particularly when biopsies are performed), bacteremia is a possibility and antibiotic regimens are indicated as needed for patients at risk for endocarditis. *Nagelhout JJ, Plaus KL. Nurse Anesthesia. 5th ed. St. Louis, MO: Elsevier Saunders Company; 2014: 1271-1272.*

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Is conscious sedation or TIVA suitable for all upper endoscopic procedures?	Conscious sedation or TIVA is suitable for the vast majority of cases, but general endotracheal anesthesia may be required for patients with strong tendencies toward upper airway obstruction or at high risk for aspiration. This should be discussed with the endoscopist as the endotracheal tube may make the procedure difficult to perform. *Longnecker DE, Newman MF, Brown DL, Zapol WM. Anesthesiology. 2nd ed. New York: McGraw-Hill; 2012: 1269.*

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